



Active Community Mental Health Consumer Presentations at Armadale Health Service Emergency Department during Business Hours Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Mental Health	Medical, Nursing, Allied Health

2. Purpose

All consumers active with Armadale Mental Health Service (AMHS) are assessed by the most appropriate mental health clinician available

Consumers should be routinely provided with information on how to access their relevant treating team during office hours and be provided with information regarding out of hours emergency contacts. All consumers should be encouraged to utilise these contacts

3. Procedure

If a consumer active with the AMHS presents to the AHS Emergency Department (ED) during office hours, the following process will be initiated.

3.1 Process

- The consumer will be triaged by the Triage Nurse at the AHS ED and the ED Mental Health Liaison Nurse (EDMHLN) will provide a mental health assessment as required.
- If the consumer is referred to the EDMHLN for assessment, the EDMHLN will identify via PSOLIS, whether they are active with any AMHS community mental health team, i.e. Assessment and Treatment Team (ATT), Clinical Treatment Team (CTT) inclusive of the Individual Community Living Support (ICLS) and Early Episode Psychosis Team (EEPT), and Older Adult Mental Health Service (OAMHS) or EMHS Crisis Resolution Home Treatment Team (CRHTT). This includes 'in progress' referrals for the ATT and OAMHS and active consumers of all programs.
- The EDMHLN will check PSOLIS for a Treatment Support Discharge Plan (TSD) /crisis plan to ascertain care and treatment being provided in community.
- The EDMHLN will contact the relevant community mental health team, EMHS CRHTT team to inform the Care Coordinator or Allocated Clinician that the consumer has presented to ED and to request collateral information.
- If the AMHS Care Coordinator is unavailable, the First on Call (FOC) clinician should be contacted for CTT consumers, the triage clinician for the DST the ATT Triage Office for ATT/EEP and the OAMHS Triage Officer for Older Adult consumers, EMHS CRHTT CNS.
- The provision of comprehensive information to the EDMHLN will be a priority.

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- The Care Coordinator, or above delegate, will determine their availability and whether their physical presence would be beneficial in the assessment, (either at the ED or at another location such as the mental health clinic). If it is mutually determined that further assessment will occur outside ED, a risk assessment must be undertaken, and documented by the EDMHLN.
- The process by which the consumer is directed or escorted to another location will be determined by the risk assessment and availability of transport options.
- If it is determined that the assessment will take place in ED, the EDMHLN will undertake this and complete a Mental Health Assessment Form (MHAF) as per [Assessment Requirements for Mental Health Consumers \(AKG-PRO-0046-15\)](#)
- Risk Assessment Management Plan (RAMP)
- Post presentation to ED, a copy of the assessment documents will be emailed to the relevant team, via AHS.EudoriaReception@health.wa.gov.au for CTT/EEP, AHS.MeadReception@health.wa.gov.au for ATT/OAMHS for the attention of the Care Coordinator or Allocated Clinician and ArmadaleCRHTTReferrals@health.wa.gov.au for the EMHS CRHTT – Armadale location. If the Care Coordinator / Allocated Clinician are on planned or unplanned leave, reception staff will forward the documentation to Triage (ATT/EEP/OAMHS) or FOC (CTT).
- The Care Coordinator / Allocated Clinician will undertake a review of the TSD and / or Crisis Plan, in conjunction with the multidisciplinary team at the next Clinical Review meeting.
- Any changes will be entered on PSOLIS and the action documented in the health record.

4. Compliance monitoring

Compliance against this policy will be monitored by the Mental Health Safety and Quality Departmental meeting. Quarterly audit to be conducted to evaluate effectiveness of policy.

5. Policy context

The following documents are required to give effect to this procedure:

The following documents are required to give effect to this policy:

- *WA Mental Health Act 2014*
- Health Department of Western Australia (HDWA) [Community Mental Status Assessments Role of Mental Health Clinicians \(MP 0099/18\)](#)
- HDWA [Mental Health Emergency and Follow Up Information on Discharge from Hospital Emergency Department \(MP 0070/17\)](#)
- EMHS [Care Coordination in Mental Health \(EMHS: 13\)](#)
- EMHS [Acute Response Mental Health Emergencies / Crisis \(EMHS: 17\)](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Clinical Treatment Team Process Manual](#)
- [HDWA Clinical Care of People With Mental Health Problems Who May Be At Risk of Becoming Violent or Aggressive Policy - MP 0101/18](#)
- [Assessment Treatment Team Process Manual](#)

- [Assessment Requirements for Mental Health Consumers \(AKG-PRO-0046-15\)](#)
- [EMHS Crisis Resolution Home Treatment Team \(CRHTT\) Service Plan](#)

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:
 - Actively involve patients in their own care
 - Meet the patient's information needs
 - Share decision-making
- Standard 5 – Action 5.10 Screening of risk
- Clinicians use relevant screening processes:
 - On presentation, during clinical examination and history taking, and when required during care
 - To identify cognitive, behavioural, mental, and physical conditions, issues, and risks of harm
 - To identify social and other circumstances that may compound these risks
- Standard 5 – Action 5.11 Clinical assessment
- Clinicians comprehensively assess the conditions and risks identified through the screening process
- Standard 5 – Action 5.31 Predicting, preventing and management self-harm and suicide
- The health service organisation has systems to support collaboration with patients, carers and families to:
 - Identify when a patient is at risk of self-harm
 - Identify when a patient is at risk of suicide
 - Safely and effectively respond to patients who are distressed, have thoughts of self-harm or suicide, or have self-harmed

8. Document contact

Enquiries relating to this procedure may be directed to:

Title: Program Manager Community Program
Department: Mental Health
Email: shannon.stuart@health.wa.gov.au

9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
---------	----------------	--------------	--------------

V3	25/10/2023	25/10/2026	Scheduled review - Minor formatting updates and remove reference to the ceased discharge support team
----	------------	------------	---

10. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	20/10/2023
Published date	25/10/2023